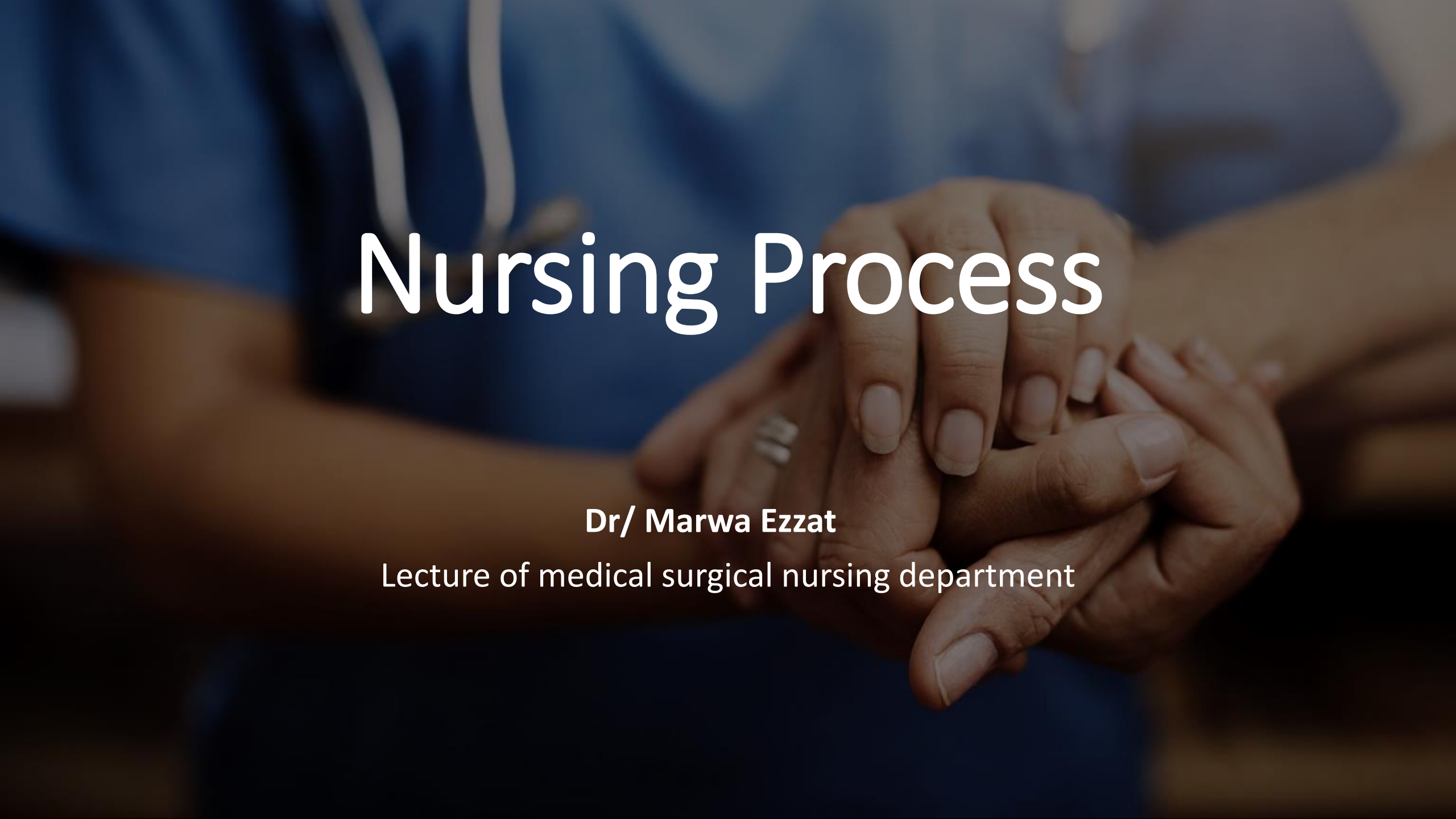
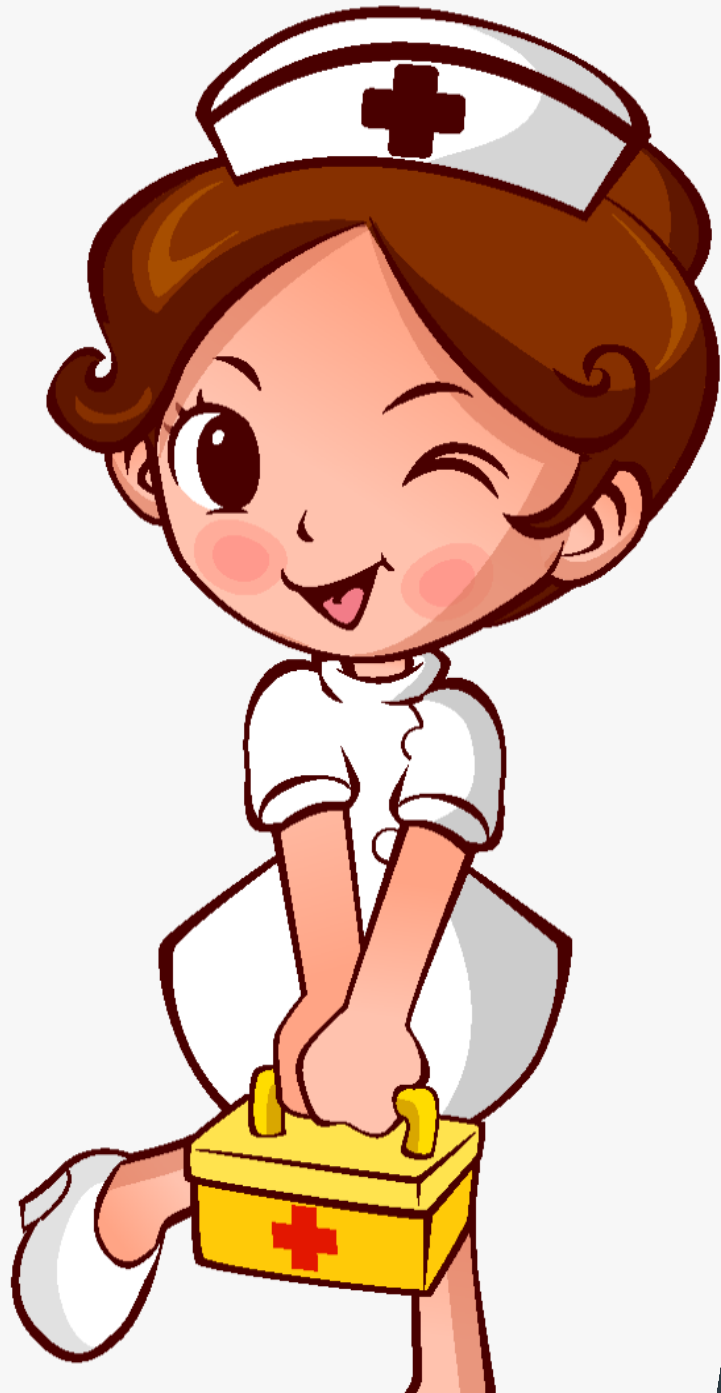




Nursing Process

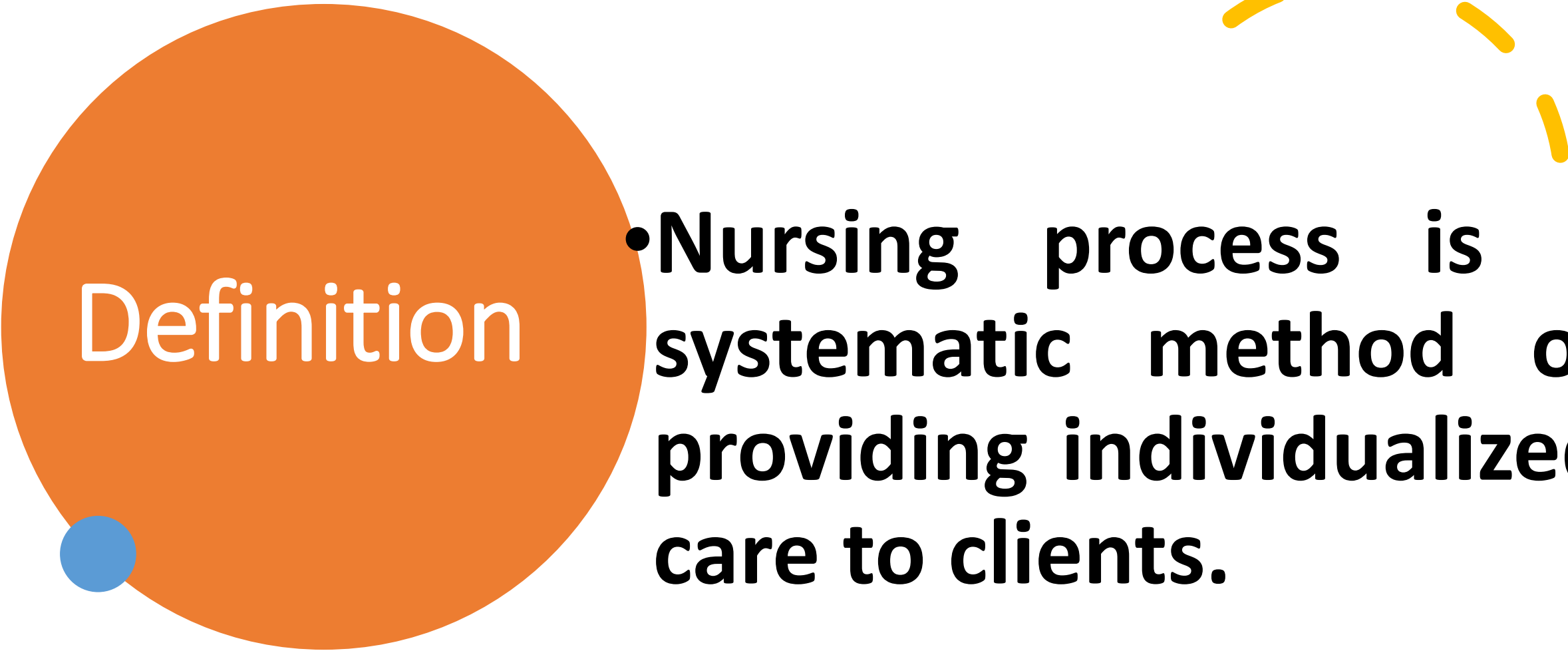
Dr/ Marwa Ezzat

Lecture of medical surgical nursing department



Outlines

- ▶ Definition of nursing process
- ▶ Purposes of nursing process
- ▶ Components of nursing process
- ▶ Characteristics of nursing process
- ▶ Steps of nursing process
- ▶ Components of a NANDA nursing diagnosis
- ▶ Comparison of nursing and medical diagnosis
- ▶ Application of nursing process



Definition

- **Nursing process is a systematic method of providing individualized care to clients.**

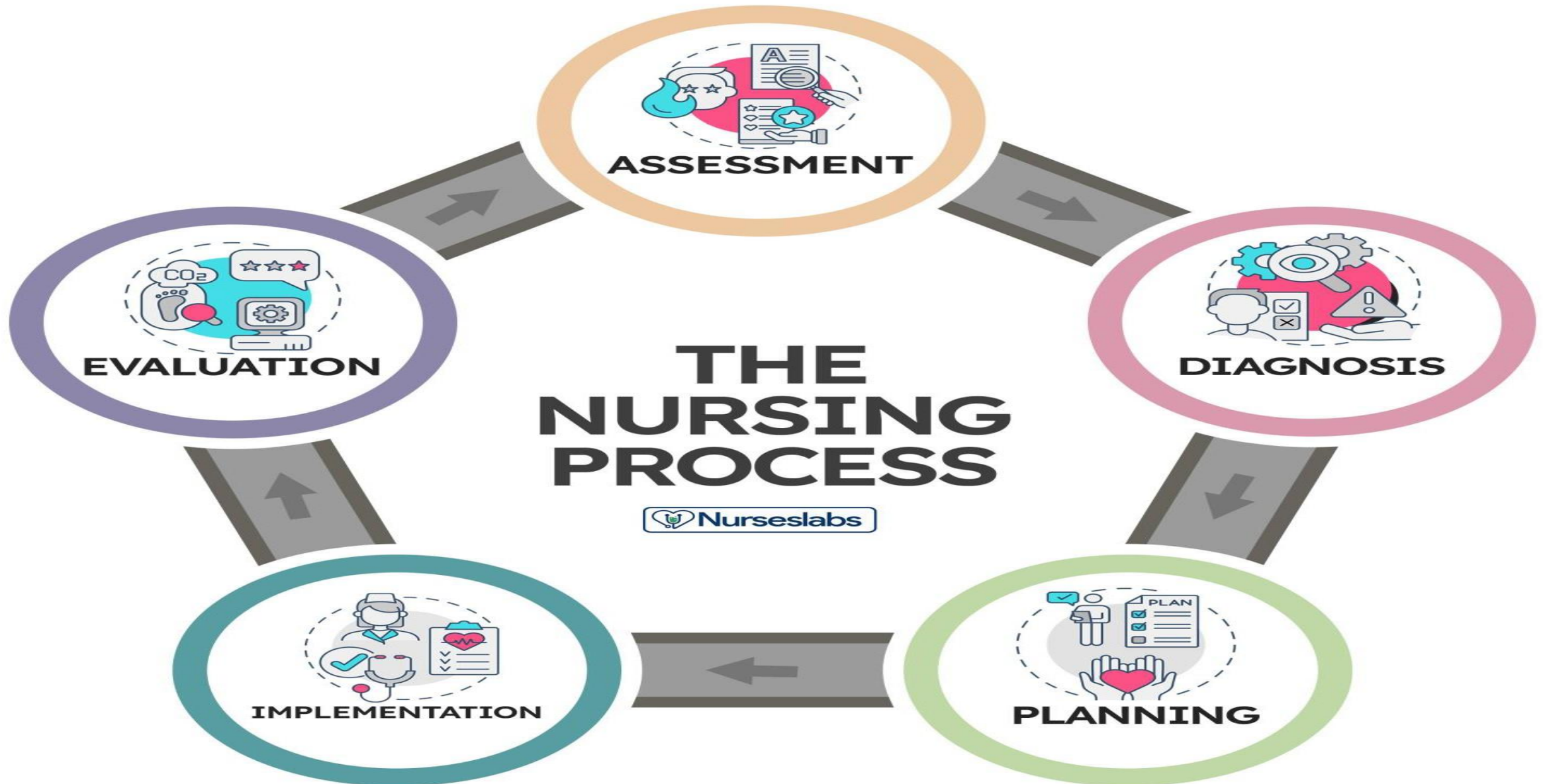
Purposes of nursing process

To identify a client's health status and actual or potential health care problems or needs.

To establish plans to meet the identified needs.

To deliver specific nursing interventions to meet those needs.

Phases/Components of nursing process



N.B.

Each phase of nursing process affects the others; they are closely interrelated.

Characteristics of nursing process



Cyclic and dynamic.



It is client centered.



Focus on problem solving and decision making.



It is interpersonal and collaborative.



Universal applicability



Use of critical thinking and clinical reasoning.



Assessment



- ❑ **Systematic and continuous collection, organization, validation, and documentation of data (information).**
- ❑ **To determine the patient's problems**

- **Organization of data;** the nurse uses a format that organizes the assessment data systematically. This is often referred to as nursing health history or nursing assessment form.
- **Validation of data;** the information gathered during the assessment is “double- checked” or verified to confirm that it is accurate and complete.
- **Documentation of data;** the nurse records client data. Accurate documentation is essential and should include all data collected about the client’s health status.

Types of assessment

1-Initial assessment: performed after admission to a health care agency

- To establish a complete data base for problem identification, reference and future comparison



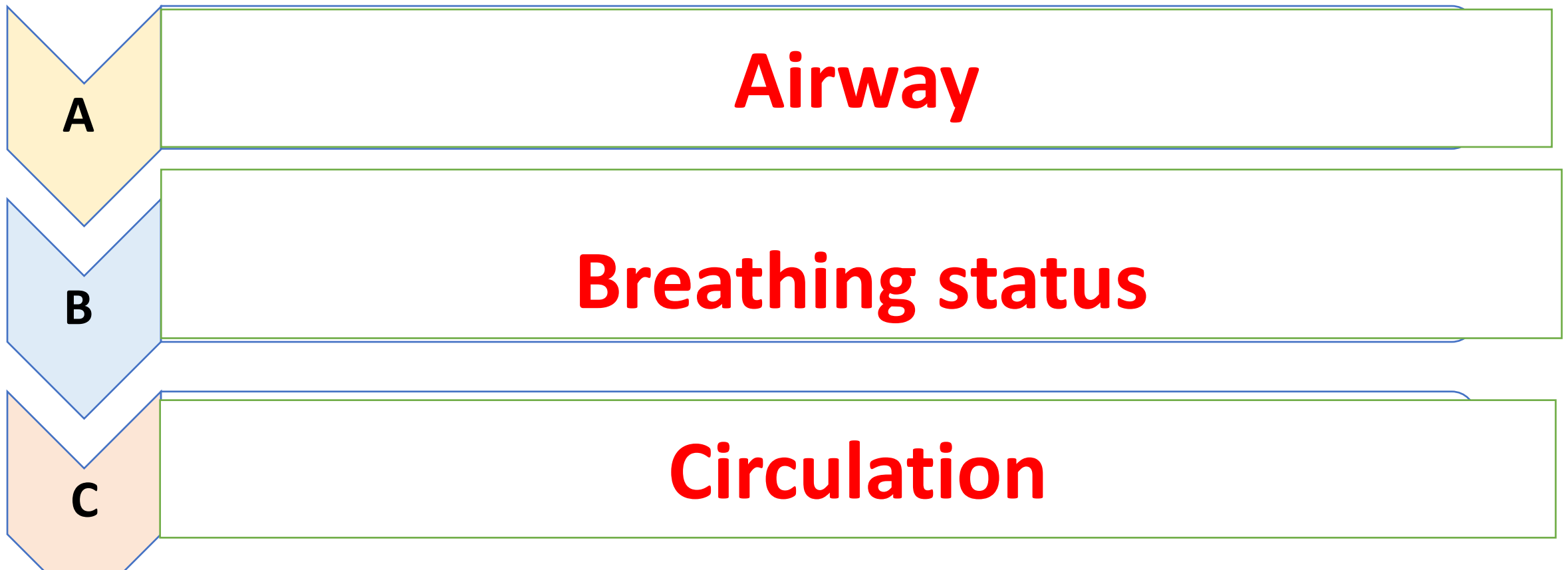
2-Problem- focused assessment To determine the status of a specific problem identified in an earlier assessment.

E.g. hourly checking of vital signs of fever patient.



Emergency Assessment

During emergency situation to identify any life-threatening situation



4-Time-lapsed assessment:

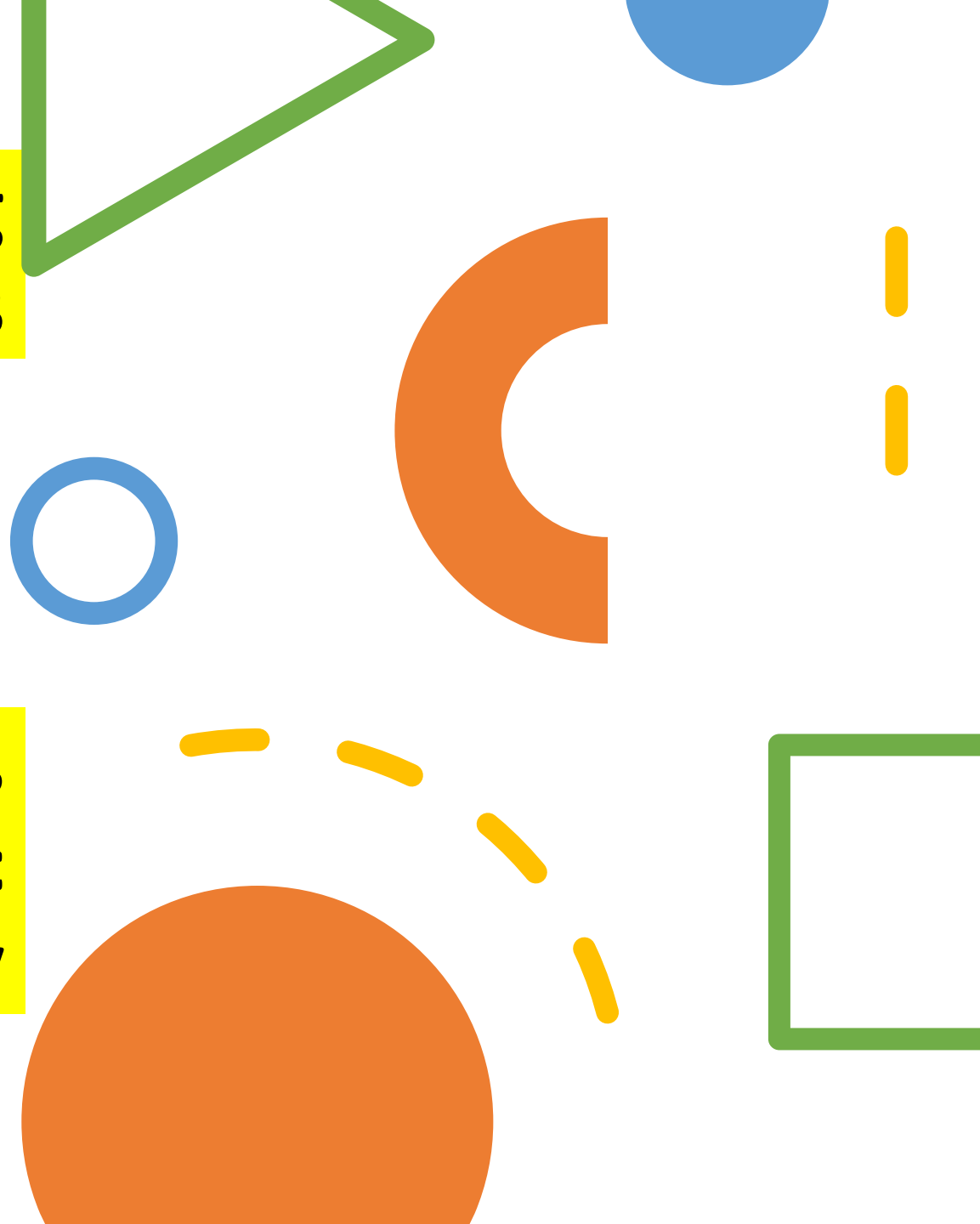
Several months after initial assessment.

To compare the client's current health status with the data previously obtained.



Collection of data

- It is the process of gathering information about a client's health status.
- It includes: Health history, Physical examination, Results of laboratory and diagnostic tests, Material contributed by other health personnel.



Physical Examination

- ❑ It is a **systematic data collection method to detect health problems.**
- ❑ To conduct the examination, the nurse uses:
 - ▶ Inspection
 - ▶ Palpation
 - ▶ Percussion
 - ▶ Auscultation.

Types of data

1-Subjective data (symptoms) or covert data:

- ❑ It can be described only by affected person.
- ❑ It includes sensations, feelings, values, beliefs, attitudes and perception of personal health status.
- ❑ E.g. Itching, pain, and feeling of worry

2-Objective data (signs or overt data)

- ❑ Detectable by an observer or can be measured or tested against an accepted standard.
- ❑ They are obtained by observation or physical examination as discoloration of the skin or blood pressure reading.
- ❑ They can be seen, heard, felt or smelled

Sources of data

1.Primary: It is the **direct source of information**. The **client is the primary source of data**.

2.Secondary: It is the **indirect source of information**.

E.g. Family members, health professionals, records and reports, laboratory and diagnostic results

Methods of data collection

► Observation

- It is gathering data by using the senses (Vision, Smell and Hearing).

► Interview: An interview is a planned communication or a conversation with a purpose.

Stages of an interview

- 1. The opening or introduction**
- 2. The body or development**
- 3. The closing**

Nursing Diagnosis

- It is the second step of the nursing process is to form diagnostic conclusions either in the form of a Nursing diagnosis.

NANDA definition of a nursing diagnosis is
clinical judgment concerning a human response to health conditions/life processes

- **North American Nursing Diagnosis Association (NANDA) define or refine nursing diagnosis**



Components of a NANDA nursing diagnosis

1. **Qualifier:** a word that provides specific meaning, category, or intensity to the core problem

❑ Describing:

- The **Nature of the problem:** (e.g., Impaired, Disturbed, Deficient)
- The **Timing:** (e.g., Acute, Chronic, Intermittent)
- The **Degree:** (e.g., Decreased, Increased, Excessive)

1. **Qualifier:** a word that provides specific meaning, category, or intensity to the core problem

□ Describing:

- The **Nature of the problem:** (e.g., Impaired, Disturbed, Deficient)
- The **Timing:** (e.g., Acute, Chronic, Intermittent)
- The **Degree:** (e.g., Decreased, Increased, Excessive)

Components of a NANDA nursing diagnosis

2. Problem (P): statement of the client's health problem
(NANDA label مسمى التشخيص)

3. Etiology (E): causes of the health problem.(related to)

7. Signs and symptoms (S): defining characteristics manifested by the client. May be one signs or symptoms or more (As evidenced by)

Signs (Objective Data): Things you can see, hear, or measure (e.g., \$BP = 160/90\$ mmHg, labored breathing, or a skin rash).

Symptoms (Subjective Data): Things the patient tells you they feel (e.g., "I feel dizzy," "My chest hurts," or "I feel anxious").

- **E.G:** Acute pain related to abdominal surgery as evidenced by patient discomfort and pain scale.
- **Modifier:** Acute
- **Problem:** pain
- **Etiology:** Surgery of abdomen
- **Signs and symptoms:** Pain scale and discomfort of patient

❑ **Acute Pain** related to **surgical incision** as evidenced by **patient's report of pain (8/10) and guarding behavior.**

❑ **Deficient Fluid Volume** related to **active fluid loss (vomiting)** as evidenced by **dry mucous membranes and decreased urine output.**

❑ **Impaired skin integrity (bed sore)** related to **physical immobility** as evidenced by **redness over the sacral area and a 2cm skin breakdown**

Status of the Nursing Diagnosis

**Actual Nursing
Diagnosis**

**Risk Nursing
Diagnosis (potential)**

**Health promotion
Diagnosis**

1- An actual diagnosis is a client problem that is present at the time of the nursing assessment. An actual nursing diagnosis is based on the presence of associated signs and symptoms. It contains 3 parts (PES)

❑ Encompassing both:

- a) **Physical:** e.g. "Ineffective Breathing Pattern (Dyspnea/Orthopnea) " OR "Acute Pain".
- b) **Psychological:** e.g. Anxiety" "Hopelessness" "Fear".
- c) **Deficient Knowledge**

Examples of physical problems:

- ▶ **Ineffective breathing pattern (Dyspnea)** related to accumulation of secretions as evidenced by nasal flaring and use of accessory muscles. (Respiratory disease)
- ▶ **Ineffective breathing pattern (orthopnea)** related to decreased cardiac output as evidenced by tachypnea (respiratory rate > 24 bpm) and use of accessory muscles. (Cardiac disease)

Examples of physical problems:

- ▶ **Altered in comfort (pain)** related to **surgical wound** as evidenced by **restlessness, facial expression, cry, catch site of pain and pain scale 8** (more than one signs and symptoms)

Examples of psychological problems:

Anxiety related to **threat to health status and upcoming surgery** as evidenced by **increased tension, restlessness, and patient stating "I am really scared about the operation"**

Examples of Deficient Knowledge:

Deficient knowledge related to **insulin injection technique** as evidenced by **patient's inability to perform a return demonstration.** مسك الحقنه

"Deficient knowledge related to **new diagnosis and lack of information** as evidenced by **patient questioning and inaccurate follow-through of instructions**

Deficient knowledge Related to **New diagnosis and lack of previous experience with insulin therapy** AEB **Patient questioning about insulin injection sites and incorrect food choices.**

2- A risk nursing diagnosis / potential : a problem doesn't exist, but the presence of risk factors indicates that a problem is likely to develop unless nurses intervene.

It must contain two-part statements ONLY

- ▶ **Problem (P)**: statement of the client's response (NANDA label)
- ▶ **Etiology (E)**:

N.B: The two parts are joined by the words ***related to*** rather than ***due to***.

Example:

- ▶ **Risk for impaired skin integrity (bed sore) related to immobility**
- ▶ **Risk for Infection**
- ▶ **Risk for Falls OR Injury**

3- A health promotion diagnosis relates to clients' preparedness to implement behaviors to improve their health condition.

Example :

- Improve nutrition and general health**
- Dental hygiene education and oral health**

NANDA nursing diagnosis:

Class: Physical Ability
Audiovisual abilities, sexual function, and mobility
Impaired bed mobility
Impaired physical mobility
Impaired wheelchair mobility
Impaired sitting
Impaired standing
Impaired transfer ability
Impaired walking

Differentiating Nursing Diagnosis from Medical Diagnosis.

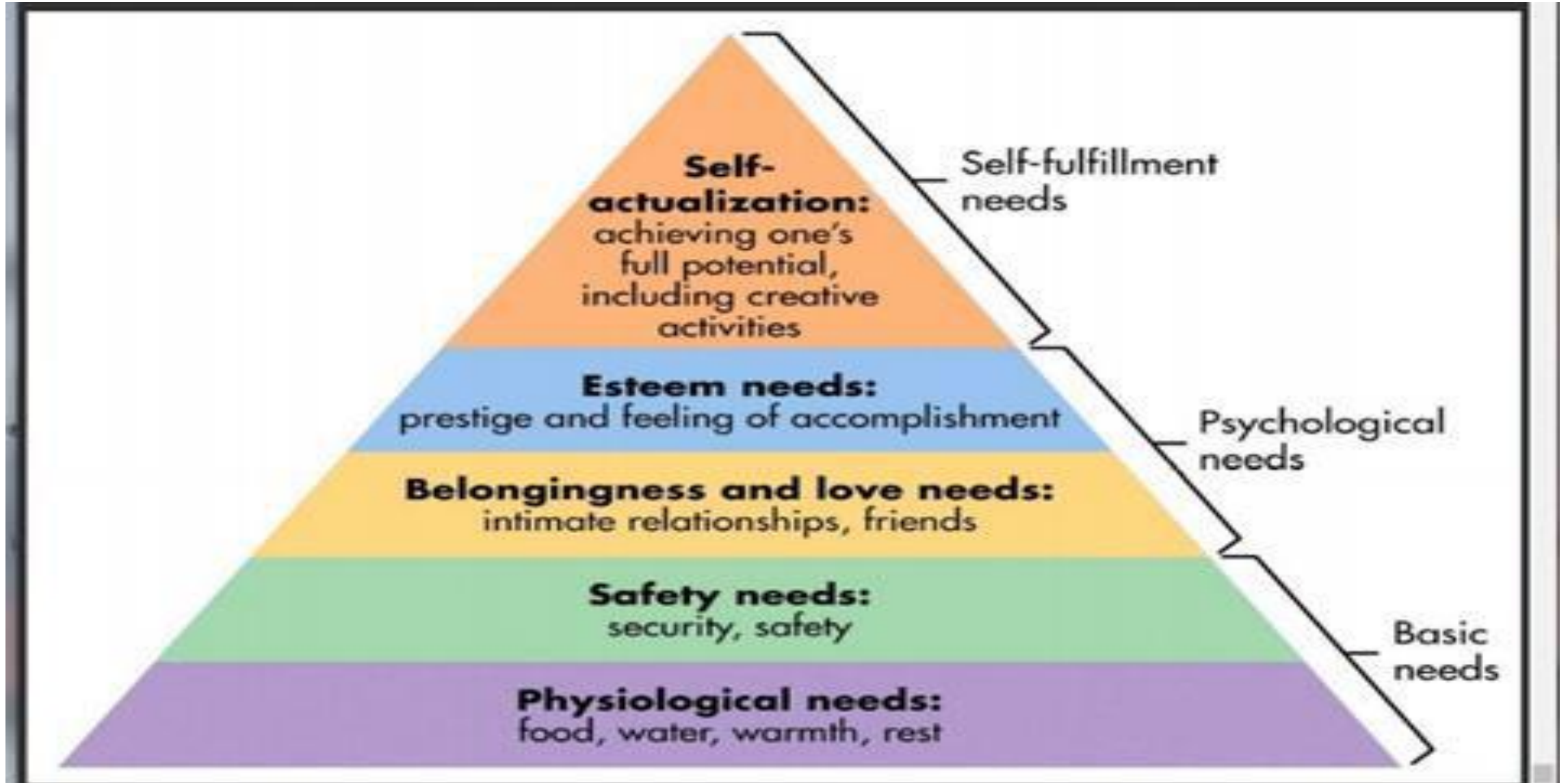
Nursing diagnosis	Medical diagnosis
A nursing judgment that made by nurse	A medical diagnosis is made by a physician
Nursing diagnoses describe the human response to an illness or a health problem Ng Diagnosis: Risk for unstable blood glucose Related to: Lack of adherence to dietary management and insulin management •Ng Diagnosis: Risk for infection. Related to: High glucose levels (Hyperglycemia) and impaired healing.	Medical diagnoses refer to disease processes e.g. Diabetes Mellitus
Nursing diagnoses may change as the client's	A client's medical diagnosis

Planning

- ❑ Planning Process includes:
 - Setting **priorities**
 - Establishing client **goals/desired outcomes**
 - Selecting nursing interventions and **activities**
 - Writing individualized nursing interventions on care plans.



Setting priorities



Planning

- Setting **priorities**

1.A - Airway

2.B – Breathing

3.C – Circulation

4.Pain



- **Examples**

A – Airway:

Priority 1: Airway Problems (Highest Priority)

These diagnoses focus on the "pipe" or the passage that air travels through. If it is blocked by secretions, a foreign body, or swelling, it is an airway problem.

- **Diagnosis: Ineffective Airway Clearance**

- **Related to: Accumulation of secretions**

- **AEB (Evidence):** Abnormal breath sounds (crackles or wheezing), ineffective cough, and excessive sputum.

- **Examples**

B – Breathing

Priority 2: Breathing Problems (Second Priority)

Once the airway is open, we look at the "pattern" and "effort" of breathing. This is about how the lungs move air in and out.

- **Diagnosis: Ineffective Breathing Pattern**

- **Related to:** Anxiety, pain, or **pulmonary congestion** (cardiac causes).
- **AEB (Evidence): Dyspnea** (shortness of breath), use of accessory muscles, and tachypnea (rapid breathing).

- **Examples**

C – Circulation

- Decreased Cardiac Output
- Inadequate Tissue Perfusion
- Fluid Volume Deficit / Hemorrhage
- Risk for Bleeding

- **Examples (physiological needs)**

Impaired Urinary Elimination

Imbalance Nutrition:

Safety & security needs

Risk for falls

Risk for infection

• **Examples (Love and Belonging Needs)**

Social isolation

Anxiety

Self-Esteem Needs

Disturbed Body Image

Deficient Knowledge

Self actualization: Spiritual Distress, Hoplessness

Establishing client goals/desired outcomes.

SMART Criteria

- ❑ **Specific:** Use clear, action-oriented verbs. Instead of "Patient will feel better," use "Patient will report a decrease in pain level."
- ❑ **Measurable:** There must be a way to quantify progress. Use scales (1–10), vital signs, or specific distances (e.g., "will walk 50 feet").
- ❑ **Attainable/Achievable:** The goal must be realistic based on the patient's current physical and mental status.
- ❑ **Relevant:** The goal should directly address the nursing diagnosis and be important to the patient's recovery.
- ❑ **Time-bound:** Every goal needs a deadline (e.g., "by the end of the shift" or "within 48 hours").

Establishing client goals/desired outcomes.



Recommended Verbs (Measurable & Observable)

1. Demonstrate, Administer (Psychomotor Skills). أنت تطلب من المريض أن يفعل الشيء أمامك لتتأكد من إتقانه.

Example: "The patient will demonstrate the correct technique for self-administering insulin by the end of the teaching session."

2. List (Cognitive Knowledge).

إذا قلت List: أنت تطلب منه سرد عدة أشياء (مثلاً: 5 أدوية).

Usage: هو فعل بسيط ومباشر جداً. يُستخدم لقياس المعرفة الحفظية عندما تطلب من المريض ذكر أسماء أدوية، أعراض، أو أطعمة.

Example: "The patient will list three signs of wound infection (redness, swelling, heat) before discharge."

Establishing client goals/desired outcomes.

Recommended Verbs (Measurable & Observable)

3. Identify / يتعرف على

عندما تطلب من المريض اختيار أو تمييز شيء يخص حالته.

Usage: لتحديد المخاطر أو الأسباب.

Example: "The patient will identify at least three personal symptoms of hypoglycemia before discharge."

4. Verbalize, Report is preferred more than express

Usage: مفيد جداً في حالات الألم أو القلق.

يُستخدم للتعبير عن المشاعر، أو شدة الألم، أو تأكيد الفهم بالكلمات.

Example: "The patient will verbalize a pain level of 3/10 or less within 30 minutes of receiving analgesic."

Establishing client goals/desired outcomes.

Recommended Verbs (Measurable & Observable)

5. Explain, Describe, Discuss

(يشرح / يفسر) يُستخدم لقياس مستوى الفهم العميق وليس مجرد الحفظ.

Usage: عندما تريد من المريض شرح "لماذا" يفعل شيئاً ما.

Example: "The patient will explain the importance of finishing the full course of antibiotics."

6. Maintain (يحافظ على)

يُستخدم للأهداف التي تتطلب استمرارية في حالة معينة أو مؤشرات حيوية.

Usage: للعلامات الحيوية، أو وضعية معينة، أو نسبة السوائل.
Example: "The patient will maintain an oxygen saturation level of 94% or higher throughout the shift."

Establishing client goals/desired outcomes.



Recommended Verbs (Measurable & Observable)

7. Perform (يؤدي / يمارس)

يشبه "Demonstrate" ولكنه يُستخدم غالباً للأنشطة الروتينية أو التمارين.

Usage:

للأنشطة اليومية (ADLs) أو التمارين العلاجية.

Example: "The patient will perform deep breathing and coughing exercises every 2 hours

8. Name: عندما تطلب تسمية محددة لشيء (اسم طبيب، اسم دواء).

Example: "The patient will name his new anticoagulant medication and its prescribed dosage before discharge."

The patient will name the primary healthcare provider to call if symptoms worsen."

Establishing client goals/desired outcomes.



Recommended Verbs (Measurable & Observable)

9. State: (يذكر / يصرّح)

عندما تطلب منه التعبير عن حقيقة أو فهم معين "بالكلام"
متى نستخدمه؟ للحقائق الطبية، المواعيد، أو أسباب القيام بشيء ما.

" *The patient will state the reason for taking his diuretic medication in the morning."

Establishing client goals/desired outcomes.

✗ Verbs to Avoid (Non-Measurable & Vague)

- ❑ Avoid these because they happen inside the patient's head. You cannot "prove" they happened without a more specific action.
- 1. **Understand:** (Too vague. How do you measure "understanding"?)
- 2. **Know:** (Abstract. Instead, ask them to state or list what they know.)
- 3. **Learn:** (Too broad. Focus on the specific skill they learned.)
- 4. **Appreciate:** (Impossible to observe or measure.)
- 5. **Think / Feel:** (Subjective. Use verbalize or a scale of 1-10 instead.)
- Improve:** (Too general. Does "improve" mean a 1% change or a 50% change?)

Establishing client goals/desired outcomes.

What are
short term goals?



Long-Term
Goals



🕒 Short-Term Goals (STG)

🕒 **Definition:** Immediate outcomes achieved in a short time.

🕒 **Timeframe:** Hours to a few days (less than 1 week).

🕒 **Focus:** Urgent symptoms and stabilization.

🕒 **Example:** "Patient will report pain reduction from 8/10 to 3/10 within 1 hour."

🕒 Long-Term Goals (LTG)

- 🕒 **Definition:** Ultimate outcomes for the patient's overall health.
- 🕒 **Timeframe:** Weeks, months, or by discharge.
- 🕒 **Focus:** Rehabilitation, prevention, and education.
- 🕒 **Example:** "Patient will demonstrate independent wound care by the time of discharge."

Examples

1. Diagnosis: Ineffective Breathing Pattern

- **Short-term Goal:**

- The patient will maintain a respiratory rate between 12–20 breaths/minute within 2 hours of nursing interventions.

- **Long-term Goal:**

- The patient will demonstrate a normal breathing pattern and clear lung sounds without the use of accessory muscles by the time of discharge.

Examples

2. Diagnosis: Acute Pain

Short-term Goal:

The patient will report a decrease in pain score from 8/10 to 3/10 within 30 minutes after receiving prescribed analgesic medication.

Long-term Goal:

The patient will remain pain-free (0/10) and perform activities of daily living (ADLs) without discomfort by the time of discharge.

Examples

3. Diagnosis: Deficient Knowledge (Diabetes Management)

Short-term Goal:

The patient will correctly perform a return demonstration of insulin injection technique by the end of the teaching session. The patient will list three signs of hypoglycemia before the end of the shift.

Long-term Goal:

The patient will maintain a stable blood glucose level (e.g., HbA1c < 7%) throughout the next 3 months of follow-up.

Examples

4. Diagnosis: Risk for Infection

Short-term Goal:

The patient's surgical wound will remain free of redness or drainage during the first 24 hours post-op.

Long-term Goal:

The patient will remain free of systemic infection (normal white blood cell count and no fever) until the wound is completely healed.

Examples

Example:

Diabetes Long-Term Goal

Goal Statement: "By the end of 3 months, the patient will demonstrate effective self-management of diabetes by maintaining blood glucose levels within the target range (e.g., HbA1c < 7%) and showing no signs of skin breakdown or foot ulcers."

Guidelines for writing nursing care plans

- ▶ **Date** and **sign** the plan.
- ▶ **Use** **standardized medical or English symbols** and **key words**
- ▶ **Be specific about** **expected timing of an intervention.**
- ▶ **Refer to the** **procedure books or other sources of information**

- ▶ **Ensure that the plan incorporates preventive and health maintenance aspects as well as restorative ones.**
- ▶ **Include collaborative and coordination activities in the plan.**
- ▶ **Include plans for the client's discharge and home care needs.**

Writing individualized Nursing interventions

Types of nursing interventions

1. **Independent interventions** are those activities that nurses are licensed to **initiate on the basis of their knowledge and skills.**
2. **Dependent interventions** are activities carried out **under the orders or supervision of a licensed physician.**
3. **Collaborative interventions** are actioning the nurse carries out in **collaboration with other health team members**

Planning

❑ Types of planning

1. **Initial Planning:** Planning which is done after the initial assessment.
2. **Ongoing Planning:** It is a continuous planning.
3. **Discharge Planning:** Planning for needs after discharge.

IMPLEMENTATION

Implementation consists of **doing and documenting the activities.**

EVALUATION

► **Evaluation** is **a planned, ongoing, purposeful** activity in which the nurse determines THE FOLLOWING:

(a) The client's progress toward achievement of goals/outcomes

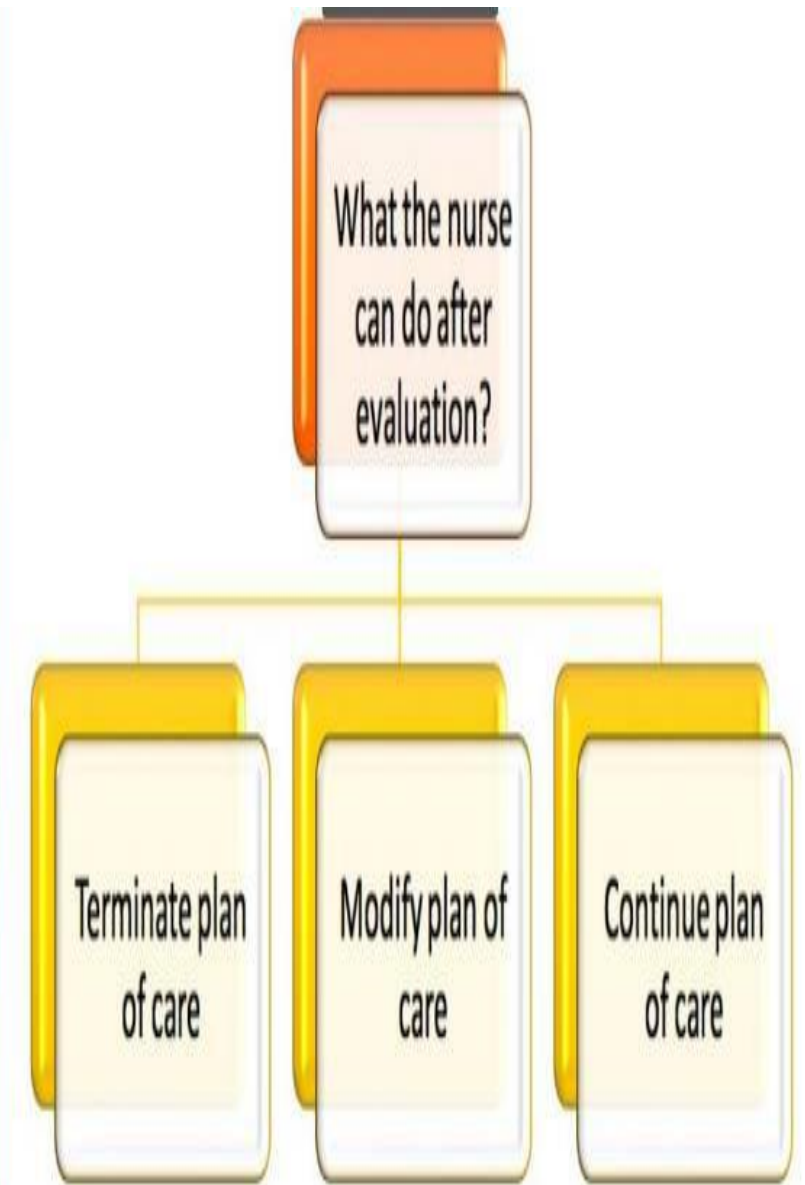
(b) the effectiveness of the nursing care plan.

The evaluation includes:

- ▶ Identifying criteria and standards
- ▶ Collecting evaluation data
- ▶ Interpreting and summarizing finding
- ▶ Documenting finding
- ▶ Care plan revision

What the nurse can do after evaluation

- ▶ Terminate plan of care or
- ▶ Modify plan of care or
- ▶ Continue plan of care







Thank you